



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صفاء شحاته عوض مصباح
المحافظة : الجيزة
الحى / المدينة : كرداسه
رقم البطاقة : 28605172301601 رقم التليفون : 01014355295
العنوان : ..
العمر (تاريخ الميلاد) : 17-05-1986 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 19-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صفاء شحاته عوض مصباح National ID: 28605172301601	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستاف
روني
١٢-٩



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مبادرة
صحة
المرأة

09/13/2026

MDT FORM

Name: <u>صباح محمد عوض مبراهيم</u>	MRN: 12036
Age: 40 Y	ID: 28605172301601
Menopausal Sta: pre	Phone: 01033402582
NRS SCORE:	Marital Status: married
BMI:	Family History: -ve
Gender:	Reason Of MDT: new
CO Morbidity: NAD	ECOG Performance Status: 0 1 2 3 4
Surgical History: CS	

Clinical Data:

BX: ILC 2
ER: 8/8 PR: 8/8 HER2: 1+ KI67: 20%

History:

Sonomammography:

CESM :RT UOQ mass 8.6 * 5.8 * 6.6 cm extending to nipple + PLN

Biomarker/Molecular

luminal A

Mets Workup:

CT CAP free of mets
bone scan requested

Recommendations:

MDT Outcome & Plan

IN VIEW OF LARGE MASS AND NON MASS TO BREAST SIZE +LUMINAL A
+ NIPPLE RETRACTION ,NO EDEMA ,NO SKIN THICKNESS
FOR PORTA-CATH ECHO NACT
AC X 4C TXT X 4C AND US /2CYCLES
THEN RT SM + ALND +/- RECONSTRUCTION



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

T3 N1 M0

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
محمد كركاش	د. مكيه ابو نوي		

5



٢٠٢٤/٠٣ ٢٨٦٠٥١٧٢٣٠١٦٠١
ربة منزل
انثى
محمود خيرى السيد عبد الحميد
البطاقة سارية حتى ٢٠٣١/٠٣/٢٣
١



اسم المريض بالإتوفارية: safaa sehata awad mosbah	كود المريض: 28605172301601
تاريخ التقرير: 2026/08/26 12:50:36 PM	العين:

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY:

- Right breast malignant mass, pathologically proved to be invasive lobular carcinoma, grade II, for pre-management evaluation.

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Moderate background enhancement
ACR: b (scattered fibro-glandular densities).

Right breast:

- **Low energy (Non contrast) images show:** upper outer quadrant (from 10 to 12 o'clock) irregular shaped spiculated hyperdense lesion associated with areolar thickening and nipple retraction.
- **Recombined (Post contrast) images show:** corresponding heterogeneously enhancing lesion of marked conspicuity measuring about 8.6 x 5.8 x 6.6 cm in its maximum dimensions, with the enhancement seen extending to the nipple.

Left breast:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

Right breast: upper outer quadrant pathologically enhancing single mass lesion reaching the nipple as described (**BIRADS 6**).

Left breast: No pathological mass or non-mass enhancement (**BIRADS 1**).

Best regards,

*Dr. Amira Shaheen, MD
Dr. Sherihan Fakhry, MD*

Patient's Name: Safaa Shehata Awad Misbāh
Date: 2026-08-02
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Known case of right breast cancer with right breast mass lesion (5.4x3.2cm) with multiple right axillary enlarged rounded lymphnodes with the largest measuring (1.8x1.2cm)
- Clear Both Lung Fields with no focal lesions or nodules & No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pericardial or pleural effusion.
- The cardiac size and great vessels appear normal.

II- ABDOMEN AND PELVIS:

- Mildly enlarged liver showing hypo yet homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Mild Splenomegaly yet with Normal CT appearance.
- Both kidneys are normal site, average size, and showing a smooth, regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal CT appearance of the pancreas, suprarenal glands, aorta, and IVC.
- The urinary bladder shows no evidence of gross masses, diverticulae, or stones.
- Normal Pelvic organs with no masses.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ✓ Right breast mass with right axillary lymphadenopathy for Sonomamography correlation.
- ✓ No detectable pulmonary nodules or masses and abdominal masses.
- ✓ Fatty Hepatomegaly & mild Splenomegaly.

Best regards,

Dr. Ayman Shawky, FEBR

Dr Ayman Shawky

Dr Ayman Shawky

Patient Name	صفاء شحاته عوض 0	Code Number	YK 276259
Sex/Age	Female 40Y	Patient ID	20699
Registered Date	13/06/2026 10:17:59	Referred By Dr.	Interventional Radiology
Validation Date	15/07/2026 11:10:53	Branch	Gustave Roussy
Reporting Date	7/15/2026, 3:25:39 AM		

Pathology Report

Pathology Report

Clinical Data: Right breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/06/2026, at 10:30 am. Cold ischemia time is 20 seconds.

Gross Examination:

- Fragmented grey-white tissue cores, ranging in size between 0.2 cm & 1.0 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive lobular carcinoma
- Histologic grade: 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- Ductal carcinoma in situ: Absent.
- Lobular carcinoma in situ: Absent.
- Microcalcification: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent.

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67%: 20%.

Conclusion:

Right breast mass, Tru-cut biopsy:

العنوان : 987 كورنيش النيل مصر القديمة - رقم خدمة : 0221277647 / 15580

Patient's Name: Safaa Shehata Awad Misbāh
Date: 2026-06-06
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH
:RESOLUTION BREAST SONOGRAPHY REVEALED**

CLINICAL SUMMARY:

Right breast palpable mass

FINDINGS:

RIGHT BREAST:

- Dense parenchyma, (ACR C).
- A large retroareolar irregular dense mass is seen with spiculated outline skin & nipple retraction.
- Complimentary U/S demonstrated the underlying heterogenous hypoechoic retroareolar lesion measuring about 5.7x4.4x3.6 cm infiltrating the nipple.
- Colour Doppler depicted the extensive abnormal vascular flow.
- No other suspicious lesions
- Left axillary level 1 two pathological lymph nodes are seen, one is enlarged with eccentric hilum and cortical thickening reaching 9 mm and the other is rounded with lost hilum measuring 8 mm.
- Free infra and supra-clavicular regions.

LEFT BREAST:

- Dense parenchyma, (ACR C).
- No mass, spiculation or suspicious micro calcifications, this was further confirmed by and U/S examination.
- Normal skin/nipple/areola complex.
- Non specific axillary nodes are noted.

CONCLUSION:

Right breast (BIRADS 5): Large mass with pathological axillary lymph nodes, for histopathological correlation.

Left breast (BIRADS 1): normal study

Best regards,
Dr. Yara Mohamed, MD

TB.NI Mx

ACR

76

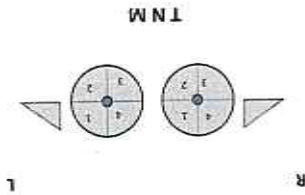
2026
06/06

			
وزارة الصحة Kingdom of Saudi Arabia		وزارة الصحة Kingdom of Saudi Arabia	
09/13/2026		09/13/2026	

MDT FORM

Name:		MRN: 12036	
Age: 40 Y		ID: 28605172301601	
Menopausal Sta: pre		Phone: 01033402582	
NRS SCORE:		Marital Status: married	
BMI:		Family History: -ve	
CO Morbidity: NAD		Reason OF MDT: new	
Surgical History: CS		ECOG Performance Status: 0 1 2 3 4	

Clinical Data:



T N M

BX: ILC 2
ER: 8/8 PR: 8/8 HER2: 1+ KI67: 20%

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Clinical Staging:

T3 N1 M0

Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4
Right Side				Left Side			

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
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اسم المريض بالإنجليزية: safaa sehata awad mosbah	كود المريض: 28605172301601
تاريخ التقرير: 2026/08/26 12:50:36 PM	الجنس:

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Best regards,

Dr. Amira Shaheen, MD

Dr. Sherihan Fakhry, MD

Patient's Name: Safaa Shehata Awad Misbāh
Date: 2026-08-02
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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- The urinary bladder shows no evidence of gross masses, diverticulae, or stones.
- Normal Pelvic organs with no masses.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ✓ Right breast mass with right axillary lymphadenopathy for Sonomamography correlation.
- ✓ No detectable pulmonary nodules or masses and abdominal masses.
- ✓ Fatty Hepatomegaly & mild Splenomegaly.

Best regards,

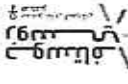
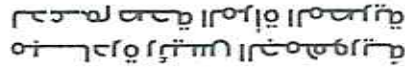
Dr. Ayman Shawky, FEBR

Dr Ayman Shawky

Dr Ayman Shawky

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09/06/2026	

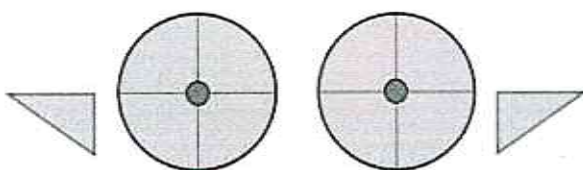
MDT FORM

Name:	Age: 40 Y
MRN: 12036	ID: 28605172301601
Phone: 01033402582	Menopausal Sta
Marital Status:	NRS SCORE:
Family History:	BMI:
Reason Of MDT:	CO Morbidity:

Clinical Data:

L

R



T N M

BX:

ER:

PR:

HER2:

KI67:

Sonomammography:

Mets Workup:

History:

40 years old neg family history, sonomammography rt breast mass 5.7 cms
 infiltrating the nipple 2 lt axillary LNs invasive lobular ca ER 8/8, PR 8/8 HER2
 1+ KI67 20% FOR CESM

Recommendations:

MDT Outcome & Plan

ct scan and bone scan no attached
 probably not done due to wt 126 kg
 to be reviewed tom in the clinic

SIGNATURE

Psychiatric	Nutrition	PATHOLOGY	RADIOLOGY	SURGERY	MEDICAL ONCOLOGY	Dr Hanan Ezzat
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Psychiatric	Nutrition	PATHOLOGY	RADIOLOGY	SURGERY	MEDICAL ONCOLOGY
SIGNATURE					

for CE-SM

MDT OUTCOME & PLAN

Recommendations:

Mets Workup:

Sonomammography:

ER:

PR:

HER 2:

KI67:

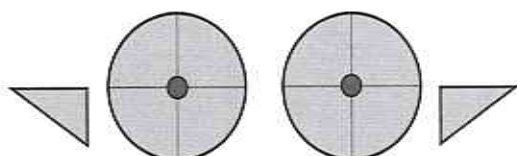
BX:

Envoire labe

R

L

T N M



Clinical Data

CO morbidity

BMI

NRS SCORE

Menopausal status

Age

Name

Handwritten signature

MRN

ID

Phone

Marital status

Family history

REASON OF MDT

MDT FORM

/ 20

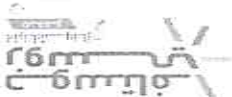
G

مركز
طبي
السرطان

مناداة رتيبة الامانة المصرية
لدعم صحة المرأة المصرية



مركز
طبي
السرطان



Pathology Report

Pathology Report

Clinical Data: Right breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/06/2026, at 10:30 am. Cold ischemia time is 20 seconds.

Gross Examination:

- Fragmented grey-white tissue cores, ranging in size between 0.2 cm & 1.0 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive lobular carcinoma
- Histologic grade: 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- Ductal carcinoma in situ: Absent.
- Lobular carcinoma in situ: Absent.
- Micrometastasis: Absent.
- Angiolymphatic invasion: Absent.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- Ki67: 20%.

Conclusion:

Right breast mass, Tru-cut biopsy:



Patient's Name: Safaa Shehata Awad Misbah

Date: 2026-06-06

Referred by: Prof. Dr. no doctor

Hospital: Goustave Roussey International Egypt

BILATERAL DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION BREAST SONOGRAPHY REVEALED

CLINICAL SUMMARY:

Right breast palpable mass

FINDINGS:

RIGHT BREAST:

- Dense parenchyma, (ACR C).
- A large retroareolar irregular dense mass is seen with spiculated outline skin & nipple retraction.
- Complimentary U/S demonstrated the underlying heterogeneous hypoechoic retroareolar lesion measuring about 5.7x4.4x3.6 cm infiltrating the nipple.
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- Left axillary level 1 two pathological lymph nodes are seen, one is enlarged with eccentric hilum and cortical thickening reaching 9 mm and the other is rounded with lost hilum measuring 8 mm.
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LEFT BREAST:

- Dense parenchyma, (ACR C).
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CONCLUSION:

Right breast (BIRADS 5): Large mass with pathological axillary lymph nodes, for histopathological correlation.

Left breast (BIRADS 1): normal study

Best regards,
Dr. Yara Mohamed, MD

Yara Mohamed

2026/08/26 12:50:36 PM : تاريخ التقرير	اسم المريض بالانجليزية: safaa sehat awad mosbah
الرقم: 28605172301601	كود المريض: 28605172301601

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY:

- Right breast malignant mass, pathologically proved to be invasive lobular carcinoma, grade II, for pre-management evaluation.

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Moderate background enhancement
ACR: b (scattered fibro-glandular densities).

Right breast:

- Low energy (Non contrast) images show: upper outer quadrant (from 10 to 12 o'clock) irregular shaped spiculated hyperdense lesion associated with areolar thickening and nipple retraction.
- Recombined (Post contrast) images show: corresponding heterogeneously enhancing lesion of marked conspicuity measuring about 8.6 x 5.8 x 6.6 cm in its maximum dimensions, with the enhancement seen extending to the nipple.

Left breast:

- Low energy (Non contrast) images No masses, asymmetry or distortion or suspicious microcalcifications.
- Recombined (Post contrast) images show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

Right breast: upper outer quadrant pathologically enhancing single mass lesion reaching the nipple as described (BIRADS 6).

Left breast: No pathological mass or non-mass enhancement (BIRADS 1).

Best regards,

Dr. Amira Shaheen, MD

Dr. Sherihan Fakhry, MD

2026/08/26 12:50:36 PM تاريخ التقرير	السن:
28605172301601 كود المريض	اسم المريض بالانجليزية: safaa sehta awad mosbah

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Best regards,

Dr. Amira Shaheen, MD

Dr. Sherhan Fathy, MD

جوسلفي
رئيسة
لجنة



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية
مصر

09/13/2026

MDT FORM

Name:	صالحه عوض محمد	MRN:	12036
Age:	40 Y	ID:	28605172301601
Menopausal Sta	pre	Phone:*	01033402582
NRS SCORE:		Marital Status:	married
BMI:		Family History:	-ve
Gender		Reason Of MDT:	new
CO Morbidity:	NAD	ECOG Performance Status	0 1 2 3 4
Surgical History:*	CS		

Clinical Data:

R

L

BX: ILC 2

ER: 8/8 PR: 8/8 HER2: 1+ KI67: 20%

History:

Sonomammography:

CESM : RT UOQ mass 8.6 * 5.8 * 6.6 cm extending to nipple + PLN

Biomarker/Molecular

luminal A

Metis Workup:

CT CAP free of mets
bone scan requested

Recommendations:

MDT Outcome & Plan

IN VIEW OF LARGE MASS AND NON MASS TO BREAST SIZE +LUMINAL A
+ NIPPLE RETRACTION ,NO EDEMA ,NO SKIN THICKNESS
FOR PORTA-CATH ECHO NACT
AC X 4C TXT X 4C AND US /2CYCLES
THEN RT SM + ALND +/- RECONSTRUCTION

Clinical Staging:

T3 N1 M0

Pathological Staging:

Right Side Left Side
Q1 Q2 Q3 Q4 Q1 Q2 Q3 Q4

TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
لجنة علاجية	د. محمد نوردي		